

Our Ref: (T) MS/VS/ [ID]

[DATE_TIME]

Clinic Letter [DATE_TIME]

[PERSON]

[ADDRESS]

West [LOCATION] Road Surgery

62 [LOCATION] Road

[LOCATION]

[LOCATION]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

Room CE145, Mailpoint 134

[LOCATION]

[ADDRESS]

[LOCATION]

Telephone:

Fax: [ID]

Dear [PERSON]

[PERSON] , [LOCATION].

DOB: [DATE_TIME] , Hospital Number: [ID] , [LOCATION] Number:

Diagnoses:

Crohn's disease with previous jejunal stricture resected [DATE_TIME]

Stage III pulmonary sarcoid (current treatment of 10 milligrams of prednisolone and 50 milligrams of azathioprine primarily for sarcoid)

This man came for review in the IBD Clinic today telling me that he has very little in the way of symptoms. He is still feeling extremely fatigued and I note that last time he saw [PERSON] his azathioprine was increased to 50 milligrams a day in the hope that this might reduce his symptoms. Unfortunately it has made no difference which raises the question of whether it is the azathioprine itself that makes him feel unwell and I am copying this letter to [PERSON] so that he can consider whether to try an alternative agent. Having said that however one might have expected him to feel better after the azathioprine was increased particularly as the original 25 milligram dosage per day has actually been dose optimised on metabolite levels (the low dosage being used because of the 100 milligrams allopurinol). I have repeated his azathioprine metabolite levels today to also help with decisions about that treatment.

In view of the fact that his GI symptoms are currently very settled I am giving him an appointment for six months' time but with a proviso that he contacts us should problems arise.

Yours sincerely

Electronic signature will go here DO NOT REMOVE

checked but not signed to avoid delay

[PERSON] FRCP

Consultant Gastroenterologist

copy to :

[PERSON]

Consultant Respiratory Physician

SG[LOCATION]